

Letter of introduction

To: Dr. Zena Gabriel, ZENA Medical, Newport Beach **From:** Scott Hansbury, Co-founder and CEO, Enyrgy Inc

Dr. Gabriel,

I looked through your treatment menu before writing this. Fraxel Dual, CO2 resurfacing, ULTRA pigment correction, Spectra, Clarity for sun spots, Thermage, and a category you call anti-aging and preventative therapy.

A good share of that list exists to correct what ultraviolet A did to somebody's skin over thirty years.

That is why I am writing.

The paradox in your own patient base

Your patients avoid the sun. The ones who follow your guidance most faithfully avoid it hardest, because for them it is not health advice, it is the entire maintenance plan behind everything else they look to you for services.

They are also, as a population, among the most vitamin D depleted people you will see. Not through carelessness. Through compliance with correct advice.

The usual answer is an oral supplement, and for most people it is adequate. For a meaningful share it is not. In research clinical trials, about one in four participants were low responders to vitamin D supplementation. They absorb it perfectly well; their cells do less with it. The only way to know which patient is in front of you is to draw a level, and most never do, because everyone assumes the capsule handled it.

You already dispense cosmeceuticals and oral medications, so this is not a foreign conversation for your practice. It is the same conversation with a measurable endpoint.

What the device is, stated accurately

The Enyrgy Vitamin D Primal Light Platform is a UVB-dominant emitter. **Over 90 percent UVB in the 280 to 315 nm band, under 10 percent UVA**, confirmed by independent spectral analysis, documentation available on request. Sunlight runs roughly the inverse, about 95 percent UVA.

That inversion is the part I think matters to you specifically. UVA is the deeper-penetrating band, and it is the one most associated with the dermal photoaging your laser calendar is built around. Per unit of vitamin D produced, this device delivers a small fraction of the UVA that sunlight does.

I am not going to tell you the remaining ultraviolet is benign, because you would stop reading. UVB is the erythemogenic band. Anyone claiming they engineered out the risky part of the spectrum is selling something.

The argument is dose. A session runs two to four minutes and produces roughly what two to four hours of midday sun would. The application reads the user's Fitzpatrick type, calculates that individual's Minimum Erythema Dose, holds the exposure below the erythema threshold, ends the session automatically, and permits one session per day. There is no timer for anyone to misjudge and no way to run it longer.

So the comparison is not ultraviolet against no ultraviolet. For the same vitamin D outcome, it is a small capped mostly-UVB exposure, versus hours of unmetered sunlight that is overwhelmingly UVA. Your patients are already choosing between those. Most are choosing neither, and taking a capsule that may not be working.

The dosing protocol was designed by Dr. Samantha Kimball, who has published over 100 peer-reviewed papers in UV therapy safety, with Dr. Bruce Hollis and Dr. William Grant.

Where I expect you to push back

Photosensitizing medication. This is the entry on our contraindication list that carries real physical risk, and your patient population is more likely than most to be on something that qualifies. The device doses on skin type and has no knowledge of what anyone takes, so the automatic cutoff does not protect against it. Our safety record is genuinely silent here, because we have never screened for those drugs and cannot claim otherwise. In your practice that question is already answerable, which is a real advantage of a clinical setting over a retail one.

Skin cancer history. Active skin cancer is a contraindication and is on the list that ships with every unit. A history of it is not, and I am not going to tell you it is fine. Published phototherapy guidance treats it as a relative contraindication, which puts it in your judgment rather than mine.

Pigmentation. You correct pigment for a living, so you will want to know whether a UVB source provokes it. The honest answer is that the dose is held below the erythema threshold, which is where the melanogenic response climbs, and that this is exactly the sort of thing I would rather you evaluate on your own equipment than take from a letter.

The nitric oxide pathway. One of the three pathways we describe is UVA-mediated photorelease, and the small retained UVA fraction is deliberate. I raise it because you would find it, and I would rather you found it here.

What we have measured, and what it does not show

Twelve weeks, five healthy adults, skin types II to IV, averaging 5.1 sessions a week. Observational, no control group.

Participant	Skin type	Start, ng/mL	End, ng/mL	Change
P1	II	38.4	86.0	+124%
P2	III	34.0	88.4	+160%
P3	III	37.0	78.0	+111%
P4	IV	47.0	88.7	+89%
P5	III	43.4	79.9	+84%
Average		39.96	84.20	+111%

Five people is a small study and we say so wherever we cite it. It is observational, the participants were highly compliant and taking sessions in office, and the sample is not diverse enough for population-level conclusions. An independent trial is targeted for Q3 2027.

What it shows is narrow: consistent sessions moved the number, in all five. What it does not show is a controlled effect size, and we do not claim one.

Answering a different question entirely, the platform has completed **more than 25,000 sessions with zero burns and zero adverse events** across 600 plus users in five countries. That is a safety record for the dosing protocol at scale, not an efficacy result, and we keep the two apart on purpose.

What it could look like at ZENA

You already deliver light therapeutically. Photodynamic therapy, LightStim, and most of your laser platform. Nobody at ZENA needs convincing that a wavelength delivered precisely is a different proposition from a wavelength delivered carelessly.

This is the vitamin D one, and it is the only light in your building that raises a number a patient can see on a lab report.

Two ways it fits, independent of each other.

In practice. Patients add it as a modality. Newport supports the top of our commercial range. The unit is \$8,950 and needs roughly four feet by four feet, with no dedicated room; most facilities put it in an alcove where someone takes a session while waiting on another treatment.

Retail, with no inventory. You already retail skin care. Patients who want a unit at home buy directly from us through your referral link and we share the revenue with you. Nothing is bought, stocked, or written down. Because the patient is the buyer of record, it is HSA/FSA eligible through Truemed even when they arrive through your link.

What I am asking for

Not a decision. Thirty minutes and a unit in your office.

We bring the platform and two of our people to Newport, set it up, and let you look at the spectral data, the dosing logic and the cutoff behavior yourself. If your read is

that it does not belong in an aesthetic practice, that is a useful answer, and I would rather have it from you than from someone less qualified to give it.

Enyrgy is a wellness device and makes no medical claims. It raises and documents vitamin D levels. Every clinical conclusion, and every screening decision, belongs to you.

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